INFORMATION TO KEEP WITH YOU

My complete treatment handbook

Capecitabine + temozolomide (capecitabine and temozolomide (CAPTEM))

Understand your treatment. Know what to expect. Keep your plan to hand.

Displayed calendar
28 days
See the active treatment phase

Treatment stages
1
Explained in the treatment reference

Regimen source
NCCP 00505
Version 5

28 days: capecitabine D1–14, temozolomide D10–14, then rest D15–28

Treatment stages

01 **CAPTEM · six planned 28-day cycles**
Different tablet days

Your team confirms the active stage and dates. The full treatment guide explains each step.

Inside your handbook

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My name _____

Hospital / oncology team _____

Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

CHAPTER 01

Understanding your treatment

IN THIS CHAPTER

Why this treatment is offered, what its medicines do, and how the agreed plan fits your care.

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Stop capecitabine and contact oncology for significant diarrhoea, painful mouth or hand-foot symptoms affecting activity, vomiting that prevents drinking, fever or sudden illness. Call urgently for new cough/breathlessness, bleeding or yellow skin/eyes. Chest pain or severe breathlessness needs emergency help. Do not restart held treatment without instructions.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

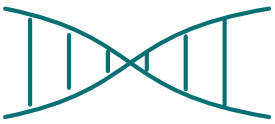
How the two medicines work

CAPTEM combines two oral chemotherapy medicines to help control an advanced pancreatic neuroendocrine tumour (NET). The team discusses the intended benefit, side effects and alternatives. NCCP identifies this pancreatic NET use as outside the medicines' licences, requiring the local consent process.

CHEMOTHERAPY TABLET

Capecitabine

Xeloda / generic



Converted to fluorouracil inside the body

Capecitabine is changed into fluorouracil, which interferes with the processes cells use to make and use their genetic instructions. This can damage cancer cells, but healthy bowel, mouth, skin and blood-forming cells can also be affected.

Tablet taken → Converted to fluorouracil → Cell growth disrupted

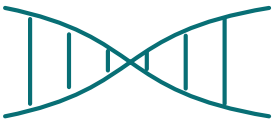
What to watch for

Stop and call for significant diarrhoea, mouth or hand-foot symptoms; chest pain needs emergency assessment.

CHEMOTHERAPY CAPSULE

Temozolomide

Temodal / generic



Damages deoxyribonucleic acid (the cell's genetic material) (DNA) in cells

Temozolomide produces DNA damage that can prevent cancer cells from continuing to grow. Blood-forming cells and other healthy tissues can also be affected, so nausea prevention, blood counts, infection awareness and liver tests are important.

Capsule taken → DNA damaged → Cell growth disrupted

What to watch for

Take the prescribed anti-sickness plan on temozolomide days; report fever, bleeding, new breathing symptoms or liver symptoms.

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

Your CAPTEM treatment

Two chemotherapy medicines with carefully separated instructions.

What the medicines do

Capecitabine is changed into fluorouracil, which interferes with the processes cells use to make and use their genetic instructions. This can damage cancer cells, but healthy bowel, mouth, skin and blood-forming cells can also be affected. Temozolomide produces DNA damage that can prevent cancer cells from continuing to grow. Blood-forming cells and other healthy tissues can also be affected, so nausea prevention, blood counts, infection awareness and liver tests are important.

Schedule and planned duration

Each cycle lasts 28 days: capecitabine twice daily on Days 1–14; add temozolomide once daily on Days 10–14; take neither anticancer medicine on Days 15–28. NCCP plans six cycles unless progression or unacceptable toxicity changes the plan.

Evidence and uncertainty

Adding capecitabine improved the time before disease progression in the trial. The final overall-survival comparison was not statistically significant. Progression-free survival is not a personal survival forecast. The study population and trial treatment duration do not replace the Irish six-cycle regimen, consent process or individual decision. Routine O6-methylguanine-DNA methyltransferase (MGMT) testing is not mandated by this source.

Understanding your particular NET

Several different features help the team choose treatment.

Where it started and where it is now

A neuroendocrine tumour begins in cells with hormone-making features. Its starting organ and the places it has spread are separate parts of the diagnosis. A pancreatic neuroendocrine tumour differs from pancreatic adenocarcinoma; a deposit in the liver may have come from another organ.

Differentiation, grade and Ki-67

The pathology report describes how organised the cells look (differentiation) and their growth activity (grade). Ki-67 measures cells showing a growth-related protein in the sample. Its percentage describes that sample, not the amount of your body affected or your chance of survival.

Grade is different from stage

Stage describes extent; grade describes tumour biology. Either can vary independently. A well-differentiated grade 3 neuroendocrine tumour and a poorly differentiated neuroendocrine carcinoma are distinct diagnoses and may need different treatments.

Functioning and nonfunctioning

A functioning tumour produces a hormone syndrome. A nonfunctioning tumour can still cause problems through growth or pressure. Somatostatin receptor uptake on a scan shows a treatment target; it does not, on its own, identify a hormone syndrome.

Your diagnosis record

Ask for one diagnosis summary: starting organ, grade, differentiation, Ki-67 result if reported, sites involved, hormone syndrome and receptor-imaging result. Keep pending results clearly marked.

Your overall plan and choices

Your priorities belong in the treatment plan.

What are we trying to achieve?

Agree your treatment aim: slowing growth, reducing tumour size, controlling hormone symptoms or preserving daily function. A scan showing no growth may be a useful result even when the tumour has not shrunk.

How treatments fit together

Your plan may use different treatments at different times, including surgery, medicines, receptor-targeted radiation or treatment directed at the liver. Ask the team to mark your own sequence and who coordinates it.

Choosing between options

Ask why the proposed option fits the tumour site, grade, receptor findings, symptoms and your health. Discuss reasonable alternatives, observation when appropriate and suitable trials. Your team explains which options are available to you.

Evidence is about groups

Studies measure different outcomes, such as shrinkage, time before worsening and survival. Ask which outcome matters for your plan and how closely the study group matches your diagnosis. No study average can predict one person’s course.



Review points and shared decisions

Agree when to review the plan and what could lead to continuing, pausing or changing treatment. Include the effects on work, caring duties, travel and the activities that matter to you.

Evidence for the pancreatic NET combination

ECOG-ACRIN E2211 compared CAPTEM with temozolomide alone.

CHAPTER 02

Your schedule, tests and appointments

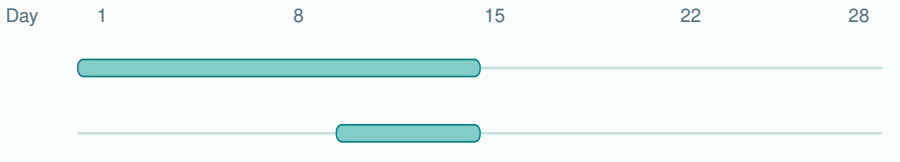
IN THIS CHAPTER

Your treatment stages, checks and practical preparations, from before treatment through follow-up.

Two medicines, different treatment days

Capecitabine + temozolomide (CAPTEM)

28-day view



This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

DIFFERENT TABLET DAYS

CAPTEM · six planned 28-day cycles

D1–9 | D10–14 | D15–28

Each cycle lasts 28 days: capecitabine twice daily on Days 1–14; add temozolomide once daily on Days 10–14; take neither anticancer medicine on Days 15–28. NCCP plans six cycles unless progression or unacceptable toxicity changes the plan.

The two medicines overlap only on Days 10–14. Six cycles are planned; dose changes, holds or stopping depend on your team's assessment.

BASELINE → TREATMENT → FOLLOW-UP

Safety tests before, between and after CAPTEM cycles

Six cycles are planned in NCCP; the team may change or stop treatment for progression or toxicity. Do not substitute a fixed scan/marker timetable from another NET treatment.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00505 v5 · current Irish CAPTEM regimen · EMA Xeloda product information and patient leaflet · EMA Temodal product information and patient leaflet

Tests in your plan

5-hydroxyindoleacetic acid (5-HIAA)

If part of your plan

WHEN

Only if requested for your hormone syndrome; the team sets the dates.

Measures a breakdown product of serotonin to help assess selected neuroendocrine tumours.

Often a timed urine collection. Follow the laboratory's food and medicine instructions.

Chromogranin A

If part of your plan

WHEN

Only when the team considers it informative for your tumour.

A selected tumour-marker result, interpreted with symptoms and scans.

Acid-suppressing medicines and other conditions can alter the result. It is not a routine test before every treatment; do not stop medicines yourself.

Selected pancreatic hormones

If part of your plan

WHEN

Only for the hormone question identified by your specialist.

Examples include gastrin, insulin or glucagon when the diagnosis or symptoms call for them.

The team names the test, preparation and review date; these are not a standard panel for every patient.

THE WHOLE TREATMENT JOURNEY

Capecitabine + temozolomide (CAPTEM): treatment and checks

Locally advanced or metastatic pancreatic neuroendocrine tumour under NCCP 00505a.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	Treatment / reviews	Between visits	Response review	After a pause or stop
Blood count / kidneys / liver NCCP 00505	full blood count (FBC), renal and liver profile	Before each cycle; liver tests also after each cycle	Earlier assessment for infection, bleeding, dehydration or symptoms	Review recovery before another cycle	Liver effects may arise weeks after the final dose
dihydropyrimidine dehydrogenase (a medicine-processing enzyme) (DPD) / hepatitis B / international normalised ratio (blood-clotting test) (INR) NCCP 00505	DPD before capecitabine unless already tested; hepatitis B surface antigen and core antibody	INR if indicated, especially with warfarin	Watch for unusually severe early toxicity	Specialist review of any DPD-related risk or interacting medicine	Follow specialist and anticoagulation plans

Care / check	Before starting	Treatment / reviews	Between visits	Response review	After a pause or stop
Symptoms / oral medicines NCCP 00505	Record usual bowel pattern, intake, mouth, skin and activity	Review adherence, worst symptoms and any doses held	Nausea plan on D10–14; symptom calls on all days	Confirm both prescriptions and the next start date	Follow persistent symptoms and return unused medicine
NET response / duration NCCP 00505	Confirm pancreatic NET, goal and six-cycle plan	Scans / hormone markers according to the consultant plan	Track symptoms; report change without waiting for imaging	Balance response, toxicity and patient goals	Discuss follow-up or next treatment after the intended course

What each check is for

1 **CAPTEM · six planned 28-day cycles**

Different tablet days

CAPTEM combines two oral chemotherapy medicines to help control an advanced pancreatic NET. The team discusses the intended benefit, side effects and alternatives. NCCP identifies this pancreatic NET use as outside the medicines' licences, requiring the local consent process.

Ask / act: Confirm the current prescription, tests and support medicines.

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Practical treatment planning

Agree the arrangements with your oncology team.

STEP 1

Dose and meal timing

The source starting schedule is capecitabine 750 mg/m² twice daily on Days 1–14 and temozolomide 200 mg/m² once daily on Days 10–14 of a 28-day cycle. Your own label gives the actual tablet/capsule numbers. Take capecitabine with food or within 30 minutes afterwards; take temozolomide fasting according to the pharmacy instructions. Swallow both whole; do not crush, cut or open them.

STEP 2

Missed or vomited doses

Do not double capecitabine or take a replacement dose after vomiting. If you miss temozolomide, follow the pharmacy's written instructions or call before taking it late. If you vomit after temozolomide, do not take a second dose that day. Never replace doses omitted because of toxicity.

STEP 3

Support medicines

Have the Days 10–14 anti-sickness prescription ready before Day 10. Ask how it fits around fasting temozolomide and meals for capecitabine. Follow the local rescue and constipation/diarrhoea plan.

STEP 4

Medicine interactions

Bring a complete medicine list. Warfarin may need extra INR checks. Tell the team about seizure medicines and recent brivudine or related antiviral treatment; dangerous interactions require specialist review.

STEP 5

Storage and handling

Keep each medicine in its own labelled original packaging and away from children. Do not cut or crush capecitabine or open temozolomide capsules. If swallowing is difficult, contact pharmacy before treatment. Return unused medicine as advised.

STEP 6

Pregnancy, breastfeeding and fertility

These medicines can harm a pregnancy and may affect fertility. Discuss contraception for patients and partners, the required duration after the last dose, fertility preservation and breastfeeding before starting; follow the medicine-specific written plan.

STEP 7

Review appointments

Before starting: blood count, kidney/liver tests, hepatitis B screening and DPD testing unless already tested. Blood count, kidney and liver tests before every cycle; liver tests also after each cycle. INR if indicated with warfarin. Liver injury can appear weeks after the last dose. Scans and hormone markers follow the consultant plan.

Your tablet, capsule and support plan

Keep a written dose and meal-time calendar.

Support medicines

Medicine interactions

Separate nausea-prevention plan

Capecitabine is minimal-to-low emetogenic, while temozolomide is moderate-to-high emetogenic in this oral schedule. The team must prescribe an oral anti-sickness plan for Days 10–14 and any rescue treatment; this is not a single low-risk infusion plan.

Mouth, hands and feet

Use the unit's mouth and skin care. Significant pain, ulcers or hand-foot symptoms may require capecitabine interruption; do not keep taking tablets just to finish the fortnight.

Diarrhoea and fluids

Ask for written diarrhoea instructions and the point at which to stop capecitabine and call. Persistent vomiting, severe diarrhoea, dizziness or inability to drink needs urgent advice.

Infection and Pneumocystis jirovecii pneumonia (PJP) prevention

The team assesses whether PJP prevention is needed under local policy, especially with steroid exposure or other risk factors. CAPTEM is not the 42-day temozolomide-plus-radiotherapy schedule; do not borrow that schedule's automatic prevention plan.

Tests, urgent symptoms and follow-up

Some effects appear after treatment days have ended.

Unusually severe early effects

DPD helps break down capecitabine's active drug. DPD testing reduces risk but does not remove it. Severe early diarrhoea, mouth soreness, infection or neurological symptoms needs immediate team assessment; do not continue waiting for the next cycle.

Making sense of tests and results

Safety tests, tumour tests and hormone tests answer different questions.

Blood tests for treatment safety

Blood counts assess oxygen-carrying cells, infection-fighting cells and platelets. Kidney and liver results help assess treatment safety. These checks are needed even when you feel well; a reassuring scan answers a different question.



computed tomography (CT), magnetic resonance imaging (MRI) and receptor imaging

CT and MRI show anatomy and tumour size. Somatostatin receptor imaging tests whether a particular receptor target is present. An fluorodeoxyglucose positron emission tomography (FDG-PET) scan measures uptake of a glucose-like tracer and is used selectively. Your team chooses the scans that can guide your care.

Hormones and tumour markers are individual

Selected markers may help answer a hormone or tumour question. Examples include 5-hydroxyindoleacetic acid (5-HIAA) for serotonin-related symptoms, chromogranin A, or a specific pancreatic hormone. They are not a routine panel for every neuroendocrine tumour. Interpret changes with symptoms and imaging.

Preparing for a test

Use the laboratory instructions for fasting, food restrictions and any timed urine collection. Some medicines affect marker results, including acid-suppressing medicines and chromogranin A. Ask the team what to do; do not stop a prescribed medicine yourself.

Heart and inherited-risk checks

Some hormone syndromes can affect heart valves, so an ultrasound of the heart may be requested. Inherited-risk assessment depends on the tumour and family history; multiple endocrine neoplasia type 1 is one example. An inherited-risk test and a test of tumour tissue answer different questions.

Who does what, and when?

For each test, write its name, purpose, booking date and result-review contact. Ask the team to set your next scan or marker date. A schedule for someone after surgery may not apply during your treatment.

CHAPTER 03

Symptoms and supportive care

IN THIS CHAPTER

Recognising changes, knowing when to call, and using your prescribed supportive-care plan.

Day 1 → Day 28: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Treatment or check	Timing in this view
Capecitabine twice daily	 **Days 1–14** With food or within 30 minutes after food.
Temozolomide once daily	 **Days 10–14** Fasting; follow the separate anti-sickness plan.
No CAPTEM tablets / capsules	 **Days 15–28** Recovery, liver tests and next-cycle assessment.
Symptoms and safety	 **Days 1–28** Serious symptoms can occur during rest days too.

Two calendars in one cycle: Capecitabine starts on Day 1; temozolomide starts on Day 10. The overlap lasts five days, then both stop for Days 15–28.

Delayed liver effects: Keep the arranged liver tests after each cycle and any follow-up after the last dose; do not assume all risks end on Day 14.

Rest days still need symptom awareness: Blood counts and other effects may worsen after the tablets finish. Continue the emergency plan throughout the cycle.

> **Why these bands stay broad:** Treatment timing is shown for orientation. Symptoms can develop on any day; no symptom band predicts an individual toxicity peak. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00505 v5 · current Irish CAPTEM regimen ↗

> **There are no guaranteed "safe days".** Follow your oncology unit's emergency instructions. Fever, shivering or suddenly feeling unwell needs urgent advice; do not wait for the next appointment. Call emergency services for collapse, severe breathing difficulty, chest pain or uncontrolled bleeding.

Between-treatment notes

Day 1–9: Capecitabine only

Take the prescribed morning and evening doses with food or within 30 minutes afterwards. Check your calendar and report bowel, mouth or hand-foot symptoms early.

Day 10–14: Both medicines; different food instructions

Continue capecitabine twice daily. Add the once-daily temozolomide dose fasting, with the prescribed anti-sickness plan. Ask the pharmacist to write practical dose and meal times; do not assume both medicines are taken together.

Day 15–28: Planned rest from both anticancer medicines

Take neither CAPTEM medicine unless the team has changed your written plan. Symptoms and low blood counts may continue. Complete liver tests after the cycle and blood tests before the next. Restart only when authorised.

Hormone symptoms and day-to-day changes

A new symptom may come from the NET, treatment or another illness.

Build a picture of what is normal for you

Keep a brief record of your usual bowel pattern, flushing, appetite, weight, pain and energy. Note new changes and when they started. This helps the team assess whether treatment, hormones or another problem needs attention.

Hormone-related symptoms

Hormone symptoms depend on the tumour: examples include flushing and watery diarrhoea, acid-related symptoms, or changes in blood sugar. An insulin-producing tumour can lower blood sugar. Ask which symptoms apply to you and whether you need a written rescue plan.

Hormone treatment is a separate part of the plan

Hormone-control medicines have their own schedule. A pause in anticancer tablets does not automatically mean pausing those medicines. Before peptide receptor radionuclide therapy, the specialist team coordinates any interruption and restart of somatostatin analogue injections.

When a change is urgent

Follow your oncology emergency plan for sudden or severe symptoms. Collapse, severe breathing difficulty, chest pain, confusion or uncontrolled bleeding needs emergency assessment. Severe diarrhoea, persistent vomiting, dehydration, fever or sudden illness needs urgent oncology advice. Do not wait for a scan, an online answer or the next clinic visit.

Procedures and hormone syndromes

Tell teams arranging an operation, anaesthetic or other procedure about a hormone-producing tumour or previous hormone crisis. Your specialist decides whether extra preparation is needed. Carry the agreed emergency instructions and current medicine list.

Eating, energy, practical help and support

Supportive care can be used alongside treatment from the start.



Food, weight and digestion

Report weight loss, persistent diarrhoea, greasy stools or difficulty eating. Different causes need different treatment. Ask for a dietitian and whether reduced digestive-enzyme function needs assessment.

Fluids and safe activity

Use your personal fluid advice, especially with diarrhoea or heart or kidney disease. Adjust activity and rest to your energy. Report dizziness, breathlessness or fatigue that limits daily tasks so treatable causes can be checked.

Emotions, relationships and quality of life

Ask for help with mood, sleep, relationships, intimacy or caring responsibilities when you need it. Symptom and palliative-care teams can support comfort and quality of life while anticancer treatment continues.

Practical help

Ask your specialist nurse or social worker about transport, employment, finances and help at home. Tell them about caring responsibilities and ask which support organisations fit your needs.

A record that works without a phone

Keep a paper schedule, current medicine list and real contact numbers together. The handbook can be read without internet access. Use the written emergency plan whenever you need urgent help.

Questions to take to the next visit

For the next visit, choose the questions that matter most: my exact diagnosis; the aim of treatment; which tests I need and why; who explains results; when to call; and what help is available.

Questions worth asking

What is the aim of CAPTEM, and how was its off-label use explained?

Ask the team to explain the expected benefit, alternatives and burdens for your pancreatic NET. NCCP identifies this use as off-label and requires the local consent process.

Has my DPD result been reviewed?

DPD testing is required before first capecitabine unless already completed. Ask the team to confirm the result and any special plan; testing does not remove every risk of severe toxicity.

Can you write exact dose and meal times for Days 10–14?

Ask the pharmacist to write a practical calendar. Capecitabine is taken with food or within 30 minutes afterwards; temozolomide is taken fasting. Do not assume the two medicines should be swallowed together.

What anti-sickness medicines should I take on temozolomide days?

Temozolomide Days 10–14 has moderate-to-high oral emetic potential. Your team must prescribe the local preventive and rescue plan and explain its timing; this guide does not select a universal drug or dose.

When are liver tests after each cycle and after the last dose?

Liver tests are needed before and after each cycle. Injury may occur weeks after the final dose, so ask for the follow-up dates and whom to call for yellow skin/eyes or dark urine.

Which symptoms mean stop capecitabine and call?

Significant diarrhoea, mouth pain or hand-foot symptoms, vomiting that prevents drinking, fever or sudden illness needs prompt advice and capecitabine interruption as instructed. Chest pain or severe breathlessness needs emergency assessment.

When will we review the six-cycle plan and response?

The NCCP plan is six 28-day cycles unless progression or unacceptable toxicity changes it. Your consultant decides response-scan timing and whether the course should continue or change.

Try explaining the plan back: Can you point out the capecitabine days, the five temozolomide days, the different food instructions and your anti-sickness plan? What symptoms mean stop tablets and call?

Pancreatic neuroendocrine tumour (NET) oral combination This guide is for the NCCP pancreatic NET CAPTEM schedule, not a temozolomide brain-tumour or radiotherapy schedule.

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

CAPTEM · capecitabine D1–14 and temozolomide D10–14

Active medicines: Capecitabine + Temozolomide

Emetogenic risk: Capecitabine oral minimal-to-low; temozolomide oral moderate-to-high

No routine prophylactic antiemetic order is populated for this phase. Agree an individual symptom-treatment plan where needed.

Additional phase-specific support

- Capecitabine is minimal-to-low emetogenic, while temozolomide is moderate-to-high emetogenic in this oral schedule. The team must prescribe an oral anti-sickness plan for Days 10–14 and any rescue treatment; this is not a single low-risk infusion plan.
- Use the unit's mouth and skin care. Significant pain, ulcers or hand-foot symptoms may require capecitabine interruption; do not keep taking tablets just to finish the fortnight.
- Ask for written diarrhoea instructions and the point at which to stop capecitabine and call. Persistent vomiting, severe diarrhoea, dizziness or inability to drink needs urgent advice.
- The team assesses whether Pneumocystis jirovecii pneumonia (PJP) prevention is needed under local policy, especially with steroid exposure or other risk factors. CAPTEM is not the 42-day temozolomide-plus-radiotherapy schedule; do not borrow that schedule's automatic prevention plan.
- A separate prescribed oral antiemetic plan is required for temozolomide Days 10–14. The source does not specify one universal drug/dose; do not substitute a parenteral low-risk bundle.
- Day-specific administration: capecitabine with food/within 30 minutes; temozolomide fasting. Omitted toxicity doses are not replaced.
- PJP prevention is a local risk-based decision for this five-day temozolomide exposure, not automatic adoption of the 42-day brain radiotherapy regimen.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

What to watch for

You do not need to know which medicine caused a symptom before calling.

1 Energy / headache

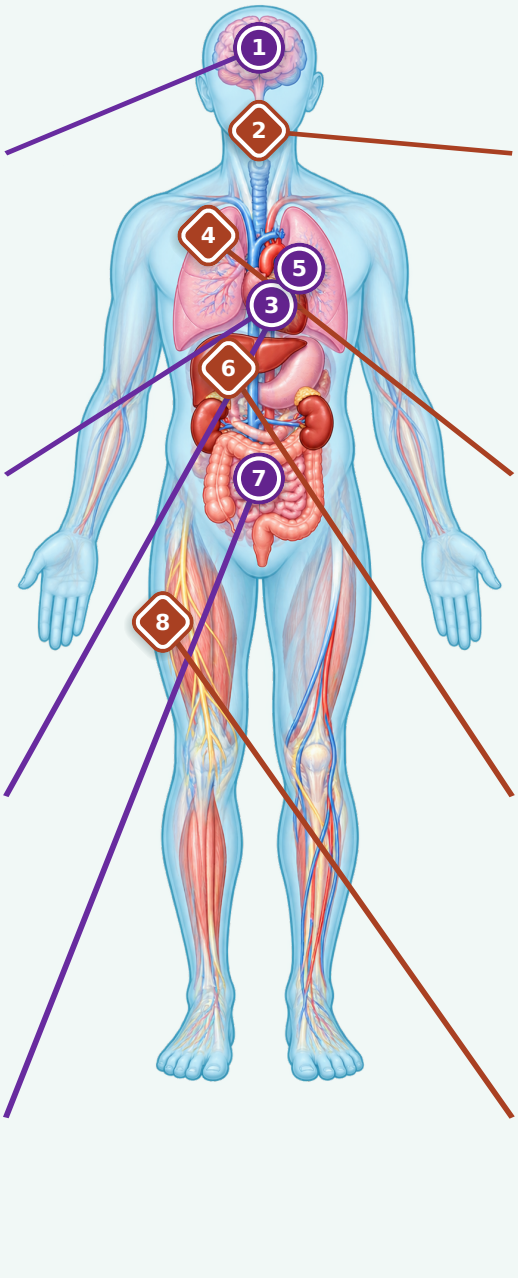
CAPTEM

Fatigue or headache can affect daily tasks. Report marked new symptoms; sudden confusion, collapse or a seizure needs emergency help.

2 Mouth / eating

Capecitabine

Mouth pain or ulcers can prevent eating and drinking. Stop capecitabine and contact the team if symptoms are significant.



3 Heart / chest

Capecitabine

Chest pain, pressure or severe breathlessness needs emergency assessment. Tell staff you are taking capecitabine.

4 Blood / infection

CAPTEM

Counts can fall, including after capsule days finish. Fever, shivering, sudden illness, unusual bruising or bleeding needs urgent advice.

5 Lungs / infection

Temozolomide

New cough, breathlessness or fever needs prompt review. The team considers infection, including PJP, and any prevention plan.

6 Liver

CAPTEM

Liver injury can develop during or after treatment. Attend tests after each cycle and report yellow skin/eyes or dark urine.

7 Bowel / nausea / fluids

CAPTEM

Diarrhoea or vomiting can cause dehydration. Take prescribed anti-sickness medicine and call for persistent symptoms or reduced urine.

8 Hands / feet / skin

Capecitabine

Painful redness, swelling or blisters can limit walking or hand use. Stop capecitabine and call when symptoms are significant.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Stop capecitabine and contact oncology for significant diarrhoea, painful mouth or hand-foot symptoms affecting activity, vomiting that prevents drinking, fever or sudden illness. Call urgently for new cough/breathlessness, bleeding or yellow skin/eyes. Chest pain or severe breathlessness needs emergency help. Do not restart held treatment without instructions.

When to get help

Call 112 / 999 for severe breathing difficulty, severe chest pain, collapse or a seizure.

Call oncology immediately: temperature 37.5°C or above, 36°C or below, shivering or suddenly feeling unwell, even with a normal temperature. Follow your hospital's temperature instructions. If you cannot reach the team, seek urgent medical assessment. Do not wait for the next appointment.

Oncology daytime: 087 12345678 · **Out of hours:** 087 87654321

EXAMPLE ONLY — team must replace both numbers before patient use.

Feasibility build. Clinical, pharmacy and patient/PPI review pending. Your team confirms your personal plan.

CHAPTER 04

Sources and further information

IN THIS CHAPTER

The references behind this information and addresses you can use to find further help.

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.

1. **Use the prescribed local supportive-care plan; independent clinical/pharmacy review pending**
   https://healthservice.hse.ie/documents/6609/505_v5_Capecitabine_and_temozolomide__zFX2DSB.pdf
2. **EMA Xeloda product information and patient leaflet**
   https://www.ema.europa.eu/en/documents/product-information/xeloda-epar-product-information_en.pdf
3. **EMA Temodal product information and patient leaflet**
   https://www.ema.europa.eu/en/documents/product-information/temodal-epar-product-information_en.pdf
4. **NCI PDQ: gastrointestinal neuroendocrine tumours (NETs) · selected marker background**
   https://www.cancer.gov/types/gi-neuroendocrine-tumors/hp/gi-neuroendocrine-treatment-pdq
5. **NCI PDQ: pancreatic NETs · hormone and digestive background**
   https://www.cancer.gov/types/pancreatic/hp/pnet-treatment-pdq
6. **ECOG-ACRIN E2211 randomised CAPTEM study**
   https://pubmed.ncbi.nlm.nih.gov/36260828/
7. **NCI: fatigue during cancer treatment**
   https://www.cancer.gov/about-cancer/treatment/side-effects/fatigue
8. **NANETS expert consensus: high-grade neuroendocrine neoplasms, 2023**
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